



OFFICIAL

Cognitive Impairment Screening Procedure

1. Applicability

Site	Operational Area	Applicable to
Armada Kalamunda Group	All areas	Medical, Nursing and Allied Health

2. Purpose

This procedure establishes the minimum standards for the screening, assessment, and management of patients with a cognitive impairment at AKG and is aligned with the Australian Commission on Safety and Quality in Health Care Clinical Care Standards.

3. Screening and Assessment

Cognitive screening and assessment allow for the detection and appropriate actions to support people with cognitive impairment (CI). CI is an overarching term referring to deficits in one or more of the areas of memory, communication, attention, thinking and judgment, and can be temporary or long-term. Dementia and delirium are the most common forms of CI in hospitalised older people, these people are at high risk of sub-optimal outcomes and adverse events, especially if their CI is not detected or identified. Younger patients can also be at risk.

All at risk patients as defined by the inclusion criteria in this policy are to receive an initial Cognitive Impairment screen using the 4AT Rapid Delirium Assessment Tool and Single Question in Delirium (SQiD):

- During presentation to the Emergency Department (ED) or as early as possible within the hospital journey
- Following direct admission to the hospital

Refer to [Appendix 1](#) for cognitive screening and assessment instruments.

Refer to [Appendix 2](#) for the AKG Cognitive Impairment Management Pathway.

Special Consideration: ICU

- In addition to the above, the Confusion Assessment Method for the ICU (CAM-ICU) can be used

Paediatric

- PARROT Chart has a section on change in behaviour / new confusion
[Observation Monitoring And Paediatric Acute Recognition And Response Observation Tool](#)

3.1 Inclusions

- All patients over 65 years of age and Aboriginal patients over 45 years
- Patients who meet the following criteria:
 - Cognitive concerns raised by others.
 - Staff concerns.
 - Pre-existing CI.
 - ⊖ A current hip fracture.
 - Severe medical illness.
 - Neurodegenerative disease.
 - Acute neurological condition.

3.2 Exclusions

- Patients with an acute brain injury requiring Glasgow Coma Scale (GCS) observations.

3.3 Emergency Department

- ED nursing and/or medical staff and/or Allied Health / Older Adult Liaison Service (OALS) will use the 4AT and SQiD to screen patients for CI and record this on the *ED Triage Form*
- Clinical staff who complete CI screening will document the results in the iSoBAR handover for the admitting ward.

3.4 Admitted patients

Following admission to the ward, nursing staff will complete the 4AT on the Multi-disciplinary Team (MDT) Patient Care Plan, or Comprehensive Continuous Care Plan (depending on area), as soon as possible. Screening should be completed within the first 10 hours following presentation to the hospital.

- If the score is 4 or more the nurse will refer immediately to the Medical Officer for further delirium assessment (to be undertaken within 2 hours) and commence a behavioural observation form (BOF).
- If the score is 1-3 the nurse will refer to the Occupational Therapist (OT) for further review (to be undertaken within 48hrs)

Any decline in cognitive functioning or a change in behaviour during the admission will be documented on the *Adult Observation and Response Chart (AORC)* as a core observation under 'New Confusion or Altered Mental State' (refer to AKG [Recognition and response to Acute Deterioration Policy](#))

- This will prompt a further 4AT screen and documentation in the MDT care plan
- Where the 4AT indicates presence of delirium (score of 4 or more) immediate referral to the MO is required for further assessment, documentation, and management and a BOF commenced.

A cognitive re-screen should also be undertaken on patients who meet the inclusion criteria following a:

- Ward transfer or bed changes on the ward.
- Patient/family request.
- New risk factor identified.
- Post incident (e.g. fall).

- Condition change.
- CI screening will be documented in the iSoBAR handover for all inter-ward transfers.

4. Ongoing Assessment

Formal cognitive assessment is to be conducted by appropriately trained MO, OTs or Psychiatric Team to facilitate assessment of CI. Tools include, but are not limited to:

- Mini-mental state exam (MMSE)
- Montreal Cognitive Assessment (MoCA – trained clinicians only)
- Confusion Assessment Method (CAM)
- Oxford Cognitive Screen (OCS)

Some Aboriginal or culturally and linguistically diverse (CALD) patients may not know the answers to some of the AMTs or 4AT questions due to their cultural background. Alternate assessment tools available for these patients are:

- Kimberley Indigenous Cognitive Assessment (KICA) - the preferred cognitive assessment tools for older Indigenous Australians (Aboriginal peoples), particularly for those living in remote and rural areas, aged 45 and older
- The Rowland Universal Dementia Assessment Scale (RUDAS) - designed to enable the translation of the assessment into other languages and to be culturally fair. It is recommended for use with those from CALD backgrounds communicating in their first language with the use of an interpreter.

Functional assessments of a patient's activities of daily living may also be used in assessing the impact of cognition on a patient's day to day tasks.

An assessment will seek input from the patient or their family/carer, GP and other primary care providers about behavioural changes such as:

- Confusion.
- Worsened concentration.
- Agitation.
- Restlessness.
- Sleepiness (including altered levels of consciousness).
- Changed communicative status.
- Cooperation with requests.
- Alterations in mood.

The treating medical team must be informed of all patients identified as having CI (via formal assessment) and consideration given to further medical assessment to identify delirium and any causative (reversible) factors. A treatment plan should be initiated as indicated and documented in the patient's medical record.

5. Management and prevention strategies

Patients with CI will be provided a set of interventions to prevent deterioration. These interventions could include:

- Medical review to identify underlying causes and correction (such as dehydration, malnutrition, and constipation)
- Infection screen, medication reconciliation review, electrolyte derangement assessment
- Referral to the Geriatricians or Clinical Psychiatry service (via eReferral where possible), depending on the severity of underlying delirium or dementia
- Regular mobilisation with assistance provided as required
- Regular fall and pressure injury risk reviews and management plan updates
- Pain assessment and management
- Regular reorientation and reassurance
- Activities for stimulating cognition
- Natural sleep remedies
- Environmental modification and support, including minimising bed moves where possible
- Completion of TOP- 5 What Matters Most
- Utilise the 'Forget Me Not Program' volunteers if available.

Refer to AKG [Delirium Clinical management Guideline \(Appendix 3\)](#)

Clinicians should discuss these interventions with the patient and their family/carer and document this in the patient's medical record. Patient centred care plans should be provided along with general educational materials (e.g. "Forget Me Not" Hospital Volunteer Program and Delirium Patient Information pamphlets).

The MDT will continue to be alert to patients displaying signs of CI interfering with their activities of daily living and must refer to the treating team and objectively document these changes within the patient's integrated notes. Patients with delirium are to be observed utilising a *Behavioural Observation Form (BOF)* (General Health) or Mental State Examination (Mental Health) for acute changes in behaviour and thinking.

The use of physical restraints should be avoided as they can increase agitation and increase the risk of injury.

6. Management of agitation in older adults with dementia or delirium

6.1 Avoiding the use of antipsychotic medicines

Antipsychotic medicines have several serious adverse effects for older people and can worsen CI.

Antipsychotic medicines should only be prescribed for patients who are distressed despite non-drug strategies, such as patients whose behavioural or emotional state poses a threat to themselves or others. Please refer to Management of Agitation in older patients' flowchart – [Appendix3](#).

If non-drug strategies are ineffective and an antipsychotic medicine is being considered:

- Refer to pharmaceutical guidelines before prescribing.
- Discuss the choice of antipsychotic medicine, the risks and benefits, dosage, and duration with the patient and carer.
- A comprehensive medication review should be undertaken with input from the ward Pharmacist.

- Commence with a low dose and closely monitor the effect before considering any dose increases.
- Limit use for as short a period as possible.
- Ensure a de-prescribing regime is considered.

7. Discharge

Planning for discharge should be in collaboration with the patient and their family/carer and ideally commence at admission.

Prior to discharge, the patient and their family/carer should receive:

- Referral to community services including contact details.
- Individualised care plan with strategies to manage diagnosed CI or how to prevent recurrence of delirium.
- General CI educational information.
- Detailed medication information (name, dose, reason and duration).
- Discharge letter to GP listing all:
 - Ongoing treatments.
 - Follow-up needed.
- Medicines, including details on those that were ceased or changed and a recommended plan for ongoing review/withdrawal.
- Possible memory clinic referral or note on GP discharge letter

7.1 Patients discharged from the Emergency Department

If the patient is to be discharged from the ED and has an 4AT score more than 1, without a previous diagnosis of CI, note the 4AT score in the discharge letter generated from the ED Information System (EDIS) and recommend-GP follow-up and referral to a Memory Clinic.

If there is known CI prior to discharge, ED medical staff must ensure it is not due to new onset delirium. If delirium is suspected, the patient is to be admitted, and cause investigated.

If the patient is transferred to another hospital, their cognitive status must be included in the verbal and written iSoBAR handover with recommendations to re-screen cognition on arrival

8. Compliance monitoring

A continuous review of patient medical records across AKG for compliance with this policy will be completed using Combined Bedside and Risk Assessment (COBRA) audit data and quarterly Delirium Clinical Care Standards audit data.

The audit reports including a quality improvement plan (if required) will be tabled at the Comprehensive Care and Recognising and Responding to Deterioration Committees.

The report will be escalated for tabling at the Patient Experience and Clinical Excellence Committee Meeting.

9. Legislative context

The following documents are required to give effect to this policy:

- HDWA

- WA Dementia Model of Care
- Other
 - [ObservationMonitoringAndPaediatricAcuteRecognitionAndResponseObservationTool](#)

10. Supporting information

The following documents support the implementation of this policy:

- AKG
- [Recognition and response to Acute Deterioration Policy](#)
- [Delirium Clinical management Guideline](#)

11. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Safe environment for the delivery of care
- Standard 2 – Partnering with patients in their own care
- Standard 5 - Minimising patient harm
- Standard 8 – Responding to acute deterioration

12. References

1. Australian Commission on Safety and Quality in Health Care. Delirium Clinical Care Standard. Sydney: ACSQHC, 2021.
2. A Better Way to Care: Safe and high-quality care for patients with Cognitive Impairment (dementia and delirium) in hospital: Australian Commission on Safety and Quality in Health Care 2nd Edition. 2019.
3. George J. (2007). Cognitive assessment in the elderly: a review of clinical methods. Quarterly Journal of Medicine 100(8): pp. 469-84.
4. Schofield, I., Stott DJ, Tolson D, McFadyen A, Monaghan J, Nelson D. Woodford HJ. (2010). Screening for cognitive impairment in older people attending accident and emergency using the 4-item Abbreviated Mental Test. European Journal of Emergency Medicine 17(6): pp 340-2.
5. Australian Commission on Safety and Quality in Health Care. (2021) Caring for Cognitive Impairment 4AT. Sydney ([4AT - All Events \(cognitivecare.gov.au\)](#))

13. Glossary

The following definitions are relevant to this policy.

Term	Definition
Cognition	The set of higher cortical brain functions that includes memory, thinking, planning, problem solving, orientation, comprehension, calculation, learning capacity, language and judgement/intuition.

Term	Definition
Cognitive Impairment (CI)	Cognitive impairment is an overarching term referring to deficits in one or more areas of cognition. It is acknowledged that the term CI is inclusive of a broader range of genetic, medical and acquired conditions and that it can affect people from any age.
Dementia	The decline in cognitive function that affects memory, judgement, attention, language and problem solving. It is usually gradual, progressive and irreversible – leading to impaired function
Delirium	An acute change in mental status characterised by a disturbance in consciousness, attention, cognition and perception that develops over a short period of time, usually over hours or days. It is usually reversible. It is a serious condition and should be treated promptly and appropriately.

14. Policy Contact:

Director Allied Health & Ambulatory Care

Enquiries relating to this policy may be directed to:

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15. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	04 12 2025	04 12 2028	Full review; significant changes through document.

16. Authorisation

This policy has been authorised and issued by:

Authorised by	Danielle Kilmurray - Director Allied Health & Ambulatory Care
Authorisation date	03 12 2025
Published date	04 12 2025

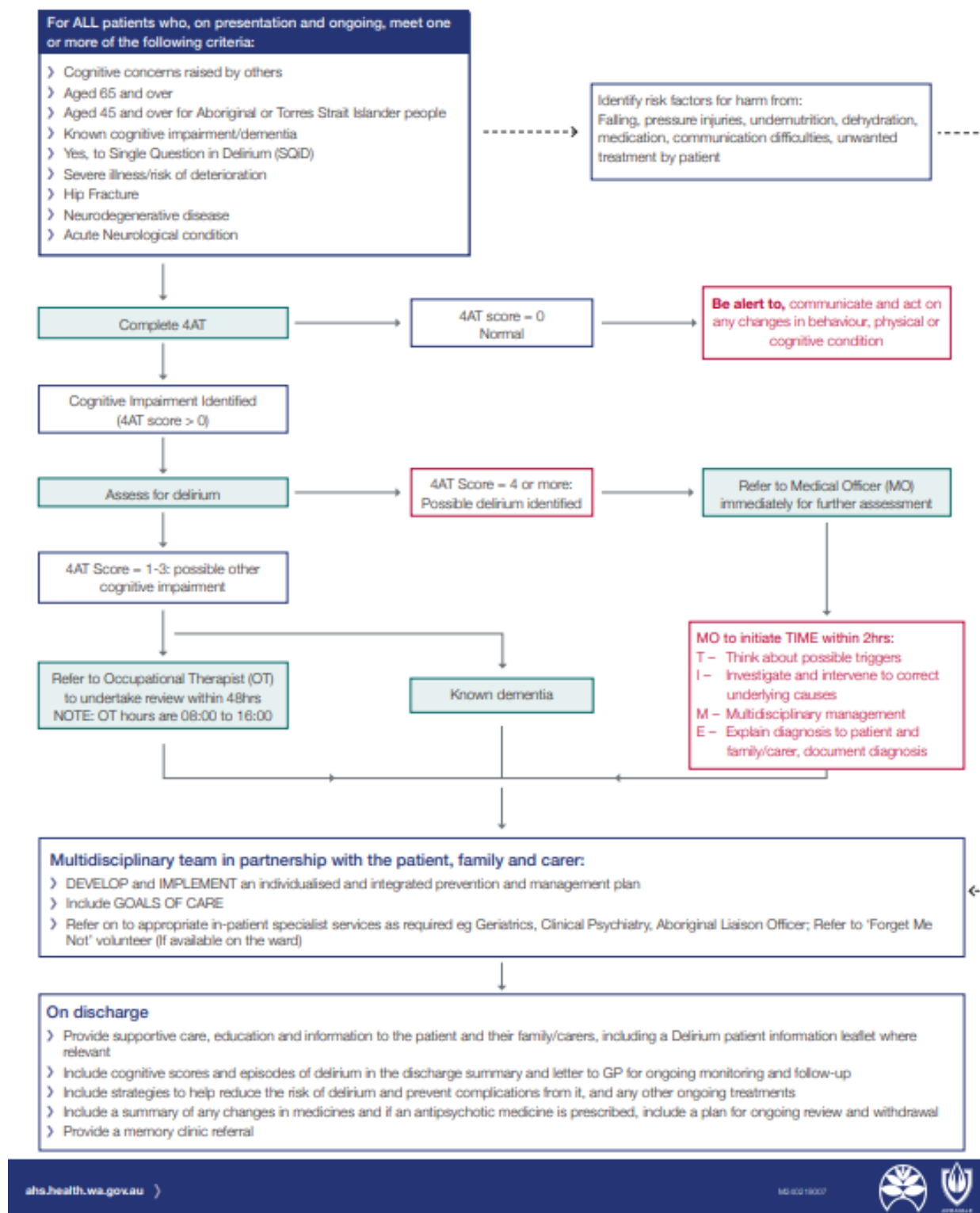
17. Appendices

17.1 Appendix 1: Cognitive screening and assessment instruments

Single Question in Delirium (SQiD)	A simple prompt question which asks a knowledgeable informant (usually a family member), “do you think (patient’s name) is more confused, sleepy or drowsy lately?” If answer is Yes, patient is identified SQiD positive which may indicate a delirium and patient should then be assessed using the CAM.
Confusion Assessment Method (CAM)	A validated diagnostic tool that allows trained clinicians to diagnose delirium.
Confusion Assessment Method for the ICU (CAM-ICU)	A validated diagnostic tool that allows trained clinicians to diagnose delirium in the ICU.
Mini Mental State Examination (MMSE)	A screening test for evaluating CI in older adults. It tests personal information, orientation, short-term memory, attention, naming, visuospatial and language domains. A score of less than 24/30 suggests CI.
Rowland Universal Dementia Assessment Scale (RUDAS)	Designed to enable the translation of the assessment into other languages and to be culturally fair. It is recommended for use with those from CALD backgrounds communicating in their first language with the use of an interpreter.
Kimberley Indigenous Cognitive Assessment (KICA)	The preferred cognitive screen for older Indigenous Australians (Aboriginal peoples), particularly for those living in remote and rural areas, aged 45 and older.
Geriatric Depression Scale (GDS)	A depression assessment tool specifically designed for older people.
Montreal Cognitive Assessment (MoCA)	A validated screening assessment for detecting cognitive impairment. It is a 30-point one page test, administered in approximately 10 minutes. Requires training and certification prior to use.
4AT – Rapid Clinical Test for Delirium	A screening tool for both delirium and cognitive impairment. The ‘months of the year backwards test’ and the ‘4-item Abbreviated Mental Test’ which are brief tests to cognitive impairment are also incorporated.
Oxford Cognitive Screen (OCS)	Designed specifically to be inclusive for stroke patients, to minimize confounds by premorbid cognitive problems and to measure commonly occurring problems after stroke.

17.2 Appendix 2: Cognitive Impairment Management Pathway

Cognitive Impairment Management Pathway



17.3 Appendix 3: Management of Agitation in older patients' flowchart

